جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : صبره سيد منظور سيد احمد
المحافظة : الجيزه **الحي / المدينة :** كرداسه **العنوان :** الجيزه
رقم البطاقة : 27302092101167 **رقم التليفون :** 01110410087 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 09-02-1973 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي زولاديكس تحت الجلد لمده 6 شهور [14163] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: صبره سيد منظور سيد احمد
National ID: 27302092101167

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

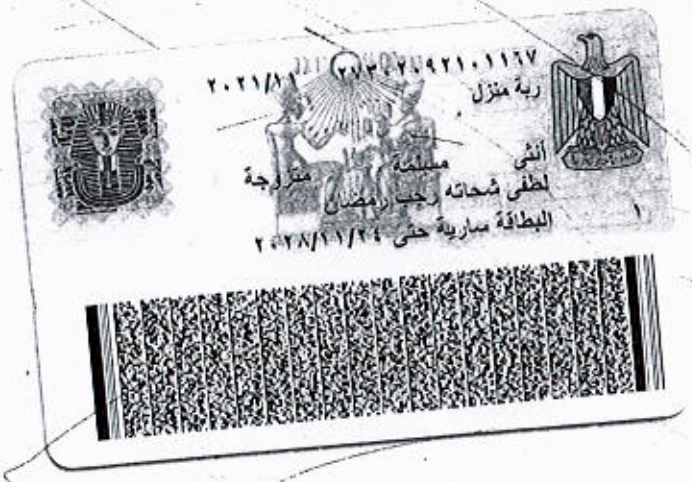
تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Goserelin (Zoladex) subcutaneous**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
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National ID - Front
[[DOCTYPE:ID_FRONT]]

Name: صبره سيد منصور	
MR. : 1036	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient presented for administration of zoladex 10.8 mg, arimidex, and supplemental therapy for a planned duration of three months.

The plan was documented as aramidex plus supplemental therapy for three months, with zoladex scheduled for 31 May 2026, pharmacy processing noted for March 2026, a three-month pause, and the medication provided at no cost.

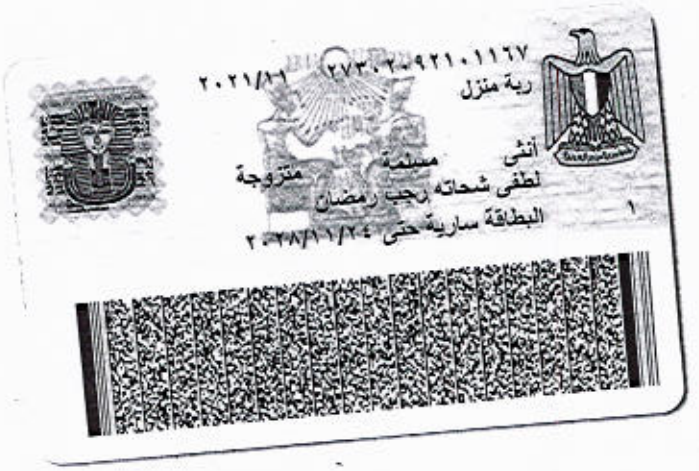
The patient has hormone-receptor-positive, HER2-negative breast cancer, staged T2N1, and has been receiving adjuvant therapy with zoladex and an aromatase inhibitor since August 2025. She reports hot flashes; physical examination is unremarkable. The current orders are arimidex, calcium, and vitamin D for three months, a zoladex injection on 23 August 2026, and arrangements for laboratory studies with a planned zoledronic acid (Zometa) administration at the next visit, which is scheduled for three months later.

تحريرا في ,

التاريخ :	19-09-2026	**الوقت :**	

الطبيب المعالج : Reham Ahmed Fathy Ibrahim

يعتمد ,



National ID - Back
[[DOCTYPE:ID_BACK]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
شبكة المراكز الطبية التخصصية
مستشفى أورام دار السلام
إدارة الأشعة

Patient Name: Sabra Sayed Manzour Sayed Ahmed	Date : 13/02/2025
Exam : C.T CHEST WITH CONTRAST	Age : 52 Year(s)
Medical ID: 147/2025	Pat. ID: 97669

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- Reported case of right breast cancer.

I Chest:

- Right breast retroareolar asymmetrical parenchymal density is noted.
- No sizable pulmonary nodules, masses, consolidation or cavitary lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- Cardiomegaly.
- Enlarged thyroid gland.

II Abdomen & pelvis:

- Average sized liver showing homogenous parenchymal attenaution. No sizable focal lesions.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, kidneys, suprarenal glands, aorta and IVC.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- Bulky uterus with IUD and anterior wall focal contour bulge mostly fiborid.
- Left iliac wing small sclerotic patch.
- Right humeral head and scapular as well as left superior pubic ramus sclerotic foci mostly bone islands.
- D3 sclerotic patch measuring 6 mm.

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
إدارة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الأشعة

Patient Name: Sabra Sayed Manzour Sayed Ahmed	Date : 13/02/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 52 Year(s)
Medical ID: 147/2025	Pat. ID: 97669

Addendum

Combined bilateral breast mammography and complementary ultrasound

Indication:

left breast palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Heterogeneously dense breasts (ACR C)
- Complementary ultrasound revealed
- Right breast:
- Right 11 o'clock retroareolar irregular spiculated hypoechoic mass lesion, with subtle vascularity detected within on color Doppler, invading the nipple, measuring 19x16mm, consistent to the palpable lump.
- No pathological regional LNs (axillary, supra and infraclavicular LNs)
- Left breast:
- No solid or cystic mass lesions
- Normal skin thickness, and nipple-areola complex.
- Nonspecific left axillary LN.

Opinion:

- Right breast 11 o'clock retroareolar malignant looking mass lesion invading the nipple (BIRADS 5) for histopathological correlation.
- No Pathological axillary LNs.
- Benign findings left breast (BIRADS 2)

Yours truly

Signature Dr. basma mohmaed

13/02/2025 12:20 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Patient Name	الأستاذه / صبره سيد منظور سيد :	Request No.	: 13004863
Patient No.	: 27302092101167	Request Date	: 12/12/2024
Path No.	: 4982/24	Verification Date	: 28/12/2024
Sex	: Female	Financial Category	: اورام دار السلام - ثقة الدولة
Age	: 51 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical data:
Right breast retroareolar mass.

Nature of specimen:
Tru cut biopsy, collected at 11/12/2024, 10:45 am.

Gross examination
Multiple greyish white tissue cores up to 0.5 cm , totally submitted.
Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9)

Microscopic examination:
Histological type: Minimally presented invasive duct carcinoma .
Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+1) Grade II .
DCIS: Focal .
Angioinvasion: Could not be detected within the limits of biopsy received.
Perineural invasion: Could not be detected within the limits of biopsy received.
Additional pathologic findings: Fibroadenomatoid changes.
Positive for microcalcifications.
IPT (Positive control showed appropriate reactivity):
ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
PR: Moderate nuclear reaction in 90% of tumor cells (score 7/8).
Her2: No membrane reaction in tumor cells (score 0).
Ki67: 20% nuclear reaction of tumor cells.

Diagnosis:

Verified by :
dr.esraa magdy

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / صبره سيد منظور سيد		
Patient No.	: 27302092101167	Request No.	: 13004863
Path No.	: 4982/24	Request Date	: 12/12/2024
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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Presidential Initiative for Women's Health - Pathology Lab

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name	: الأستاذه / صبره سيد منظور سيد	Request No.	: 13004863
Patient No.	: 27302092101167	Request Date	: 12/12/2024
Path No.	: 4982/24	Verification Date	: 28/12/2024
Sex	: Female	Financial Category	: لام - نفقة الدولة
Age	. 51 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa moha

Pathology Report

Right breast retroareolar mass, tru cut biopsy:
Minimally presented invasive duct carcinoma, Grade II .
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Negative (score 0).
- Ki67: Positive in 20% of tumor cells.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% ar 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membran staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2 based on circumferential membrane staining that is incomplete and/or weak/moderate ar within < 10%of the invasive tumor cells or complete and circumferential membrane staini that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumfere membrane staining that is incomplete, intense < 10% of tumor cells.

Prof.Gamal Abdelatty
Dr.Rofaydah Abdallah

Verified by :
dr.esraa magdy

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Presidential Initiative for Women's Health - Pathology Lab

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name	: الأستاذه / صبره سيد منظور سيد	Request No.	: 13004863
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Prof.Gamal Abdelatty
Dr.Rofaydah Abdallah

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Pathology Report
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